

IPD

IPD is a cancer drug treatment. It is used to treat a type of blood cancer called myeloma.

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What is IPD?

IPD is a combination of the following cancer drugs:

- isatuximab – a [targeted therapy drug](#)
- pomalidomide – a targeted therapy drug
- dexamethasone – a [steroid](#).

It is used to treat a type of blood cancer called [myeloma](#).

It is best to read this information with our general information about [cancer drug treatments](#) and the [type of cancer](#) you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

How IPD is given

You have isatuximab as a drip into a vein (intravenously) or as an injection under the skin (subcutaneously). You usually have this in a chemotherapy day unit or clinic as an outpatient. You take pomalidomide as capsules. You take dexamethasone as tablets.

During your course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- chemotherapy nurse or specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During your course of treatment, you will have regular blood tests. This is to check that it is safe for you to have treatment.

You will also meet with your doctor, nurse or pharmacist. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your cancer drugs.

Your course of treatment

You usually have a course of several cycles of treatment over a few months. Each cycle of IPD takes 28 days (4 weeks):

- You usually have isatuximab once a week in cycle 1. From cycle 2, this changes to once every 2 weeks.
- You usually take pomalidomide capsules every day for 3 weeks (21 days). Then you have no pomalidomide for 1 week.
- You usually take dexamethasone tablets once a week.

Your cancer team will discuss your treatment plan with you. They may give you a copy of the plan to take home.

They will give you the capsules and tablets to take home. They may give you capsules and tablets of different strengths. Always take these exactly as they tell you to. This is important to make sure they work as well as possible for you.

Your cancer team may also give you anti-sickness drugs and other medicines to take home. Take all your capsules or tablets exactly as they tell you to.

Having isatuximab

You will have medicines before having isatuximab. This is to reduce the chance of an allergic reaction.

If you have isatuximab as an injection under the skin (subcutaneous), your nurse will give the injection into your tummy (abdomen). They will change the exact place they give you injections to reduce any irritation. Tell them if the skin in the injection area becomes painful or swollen.

If you have isatuximab as a drip into a vein (intravenous infusion), you have the drip through 1 of the following:

- a [cannula](#) – a short, thin tube the nurse puts into a vein in your arm or hand
- a [central line](#) – a fine tube that goes under the skin of your chest and into a vein close by
- a [PICC line](#) – a fine tube that is put into a vein in your arm and goes up into a vein in your chest
- an [implantable port](#) (portacath) – a disc that is put under the skin on your chest or arm and goes into a vein in your chest.

Taking pomalidomide capsules

You can take pomalidomide with or without food. Swallow the capsules whole with a glass of water. Do not chew, open or crush them. Try to take them at the same time on the days you take them.

If you forget to take the capsules, take the missed dose as soon as possible within the same day. If a full day has gone by, do not take a double dose. Contact the hospital on the 24-hour number you have been given for advice.

Other things to remember about your capsules:

- Do not take a broken capsule. If you notice powder in the packaging or that a capsule is broken, contact the hospital for advice.
- Wash your hands after taking your capsules.
- Other people should avoid direct contact with the drugs. If someone helps you to take your capsules, they should wear disposable gloves. If they are pregnant, they should not touch the capsules.
- Keep the capsules in the original package and at room temperature, away from heat and direct sunlight.
- Keep them safe, somewhere children cannot see or reach them.
- If you are sick (vomit) just after taking them, do not take an extra dose. Take your next dose at the usual time.
- Do not throw away unused capsules. Return them to your cancer team at the hospital.

Taking dexamethasone

Steroids can irritate the stomach lining. If you have indigestion or pain in your tummy, tell your doctor, nurse or pharmacist. They can prescribe drugs to help reduce stomach irritation.

Take your dexamethasone with food to help protect your stomach. Some steroid tablets are coated to help reduce irritation.

Some people find it more difficult to sleep while taking steroids. Taking your dexamethasone in the morning may help.

Preventing pregnancy while taking pomalidomide

You must not become pregnant or make someone pregnant while taking pomalidomide. This is because it may cause severe abnormalities in developing babies.

You will take part in a pregnancy prevention programme during treatment. Even if you do not think you are at risk of getting pregnant or making someone pregnant, you will be included in this programme.

Your doctor, nurse or pharmacist will give you information about the programme, and the risks of pomalidomide and pregnancy. They will ask you to sign a consent form to show that you understand the information.

Pregnancy tests and contraception

If you are of childbearing age and able to get pregnant, you will have a pregnancy test before starting this treatment. The pregnancy test is repeated every 4 weeks during treatment, and 4 weeks after treatment finishes.

You must use an effective form of contraception, such as an implant, injection or the progesterone-only pill. You should not use the combined oral contraceptive pill. It increases your risk of developing blood clots.

You must use contraception to prevent pregnancy:

- for 4 weeks before treatment
- during treatment
- for 4 weeks after treatment finishes.

If you think you may be pregnant at any time during your treatment, contact your doctor, nurse or pharmacist straight away.

Contraception to prevent making someone pregnant

Pomalidomide can pass into the semen that carries sperm. During treatment and for a week after treatment finishes, you must not:

- make someone pregnant
- donate sperm.

You can protect partners by using a condom during sex. During treatment and for a week after treatment finishes, you must use a condom during sex with anyone who is:

- pregnant
- of childbearing age and able to get pregnant.

If a partner thinks they might be pregnant during your treatment, contact your doctor, nurse or pharmacist straight away.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Side effects while treatment is being given

Some people have side effects while they are having isatuximab or shortly after they have it.

Allergic reaction

Some people have an allergic reaction while having this treatment. The first infusion is the most likely to cause a reaction, so it is usually given more slowly than later treatments. Before treatment, you will have medicines to help prevent or reduce any reaction.

Signs of a reaction can include:

- feeling hot or flushed
- shivering
- itching
- a skin rash
- feeling dizzy or sick
- a headache
- feeling breathless or wheezy

- swelling of your face or mouth
- pain in your back, tummy or chest.

Your nurse will check you for signs of a reaction during your infusion. If you feel unwell or have any of these signs, tell them straight away. If you do have a reaction, they can treat it quickly.

Sometimes a reaction happens a few hours after treatment. If you develop any of these signs or feel unwell after you get home, contact the hospital straight away on the 24-hour number.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Risk of infection

This treatment can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an [infection](#). A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. Your doctor may give you antibiotics and other drugs to take during this treatment. These may help reduce your risk of getting an infection.

But it is still important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) often, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

Your white blood cell count will usually return to normal before your next treatment. You will have a blood test before having more treatment. If your white blood cell count is low, your doctor may delay your treatment for a short time, until your cell count increases.

You may be given a drug called [G-CSF](#). This encourages the body to make more white blood cells. You have it as a small injection under the skin.

Bruising and bleeding

This treatment can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

- nosebleeds
- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)
- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Loss of appetite

This treatment can affect your appetite. Don't worry if you do not eat much for 1 or 2 days. But if your appetite does not come back after a few days, or if you are losing weight, tell your doctor, nurse or pharmacist. They can give you advice. They may give you food or drink supplements. Or they may suggest changes to your diet or eating habits to help.

Diarrhoea

This treatment may cause [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Feeling sick

You may feel sick during this treatment, but this is usually mild. Your doctor, nurse or pharmacist may prescribe you anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to. [If you feel sick](#), take small sips of fluid often and eat small amounts regularly. It is important

to drink enough fluids.

If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Blood clot risk

This treatment can increase the risk of a [blood clot](#). Your cancer team will give you drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call 999 if you have:

- chest pain
- difficulty breathing.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Tumour lysis syndrome (TLS)

Some people are at risk of developing a condition called tumour lysis syndrome (TLS). The risk is highest at the beginning of treatment. TLS happens when treatment makes large numbers of cancer cells die and break down quickly. This releases lots of waste products into the blood and can affect

the kidneys and heart.

TLS can be prevented or treated. You will have regular blood tests to check for TLS. You may have:

- extra fluids through a drip
- medicines such as allopurinol tablets or [rasburicase](#) through a drip.

Drinking at least 2 litres (3½ pints) of fluid a day will also help.

Effects on the heart

This treatment can affect how the heart works. You may have [tests](#) to check how well your heart is working. These may be done before, during and after treatment.

If the treatment is causing [heart problems](#), your doctor may change the type of treatment you are having.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat (palpitations)
- swollen feet and ankles.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call **999** if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Second cancer

This treatment can increase the risk of developing a second cancer. The benefits of treatment usually far outweigh this risk. Your doctor, nurse or pharmacist can give you more information.

Other important information

Blood transfusions

This treatment can affect the result of a blood test called the indirect Coombs test. You have this test before a blood transfusion. It matches you to a donor's blood.

Before you start IPD treatment, you will have the blood tests you need for a blood transfusion. Your doctor may also give you a card to carry that explains the treatment you are having.

If you need a blood transfusion during treatment or for 6 months after, tell the person arranging it that you have had this treatment. You can show them the card your doctor has given you.

Hepatitis B reactivation

Tell your doctor if you have ever had hepatitis B (a liver infection). This treatment can make it active again. Your doctor, nurse or pharmacist will talk to you about this and will test you for it before you start your treatment.

Shingles

You may be more likely to develop a viral infection called shingles during this treatment. Your doctor may give you medicine to help prevent shingles.

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
-

use illustrations to explain text

- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help